Hope Hospitals Creation Date 01/01/2017 Released Date 11/01/2017	**Manual of Operations**	
	COP/SOP/ 05	**Restraints Policy**
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A. Purpose: To provide written guidelines for managing patients under restraints

B. Scope : Covers patients declared to be cared under restraint by the treating Consultant /Medical Officer.

C. Definition:

Restraint: Restraint may be defined as a method which restricts the movement of the whole or a portion of patient's body for the purpose of preventing intentional harm to self or others.

D. Policy:

i. It is the policy of Hope hospital that patient may be secluded in accordance and compliance with NABH standards and applicable law.

ii. Restraint shall not be used for staff convenience or as a form of punishment. All restraints will be conducted in humane, safe and effective manner without intent to harm or create undue discomfort to the patient.

iii. Restraint are to be used only in an emergency , when there is imminent risk of an individual physically harming himself or herself or others ,including staff.

iv. When the patient is violent and is likely to injure him or others in the vicinity physical restraints needs to be applied with proper precautions.

v. Similarly when found necessary chemical restraints measures may also be used. Such a patients needs to be monitored more frequently by the nurse-in-charge and periodically by the Doctor on duty

vi. Physical Restraints should be soft and not injuring the parts of the patients

vii. For those patient who have a history of physical abuse and/or sexual abuse ,special attention is paid to psychological risk during restraint.

viii. Any patient who needs to be observed and cared under restraints is to be so declared by the treating Consultant/Medical Officer with the reasons for such restraints.

ix. Any complications / problems arising out of such restraints should be communicated with the Consultant in-charge and his / her further advice sort.

x. Patients family members has to be kept informed about the need for putting the patient under restraint ,patients condition etc.

E. <u>Procedure</u>:

1. **Restraint shall be limited to** :
    1. Preventing and reducing serious, destructive /damaging actions by the patients such as :
        a. Threatening to self
        b. Threatening to others
        c. Threatening to staff
        d. Assaultive to self
        e. Assaultive to other patients
        f. Assaultive to staff , etc

2. **Patients rights regarding restraints** – The patient has a right to expect that, prior to the use of restraint , the staff will assist him/her in regaining control by :

    i. Less rectrictive interventions including :
    ii. a.Verbal conversation to descalate the patient.
    iii. b.Mediating in disputes
    iv. c.Offering PRN medications

    v. PRN medication to be offered PO initially
    vi. IM PRN medication will be used if patient refuses PO medication and poses a threat to self or others.
    vii. It is expected that all restrictive non-physical interventions will have been tried prior to utilizing restraints , when possible.

3. **Physicians responsibility** :

    a. The physician must see and evaluate the need for restraint within one hour after the initiation of restraint.
    b. A nurse, when physician is not immediately available, may asess the situation and initiate the 'emergency' use of restraint.
    c. The physician will write a written order on the doctor's order sheet designating the application of restraint and the device for the same.
    d. The order for use of restraint may not exceed four hours per order.
    e. When a written order cannot be obtained immediately, verbal order for restraint is to be entered doctors order sheet by the nursing staff.
    f. All verbal orders for restraint must be counter signed by a physician within **<u>one</u>** hour.

    g. Within one hour the physician shall evaluate the patient to determine

i. whether continued restraint is appropriate and document this evaluation in
ii. the patients progress note.

h. If restraint is continued beyond four hour a physicians personal reevaluation is necessary.

F. Nurse's Responsibility:

1. In an emergency situation when a physician is not readily available, the nurse may assess the situation for appropriateness, and if it is deemed appropriate, authorize an 'emergency use of restraints" for upto one hour.

2. Make a clinical determination as to whether additional staff are necessary to place the patient in restraints to ensure the safety of the patient and staff.

3. Explain to the patient the reason for restraint and the explicit behavioral criteria for termination.

4. Contact a physician and obtain an order (written/verbal) for use of restraints within the first one hour of the patient being restrained.

5. Assess the patient to assure that all medical needs are addressed during restraints.

G. Procedure to be observed pre and post application of restraint :

1) Inspect the bed, chair, restraints and any other equipment which will be used in he restraint process for safety prior to use.

2) Explain to the patient, the staff's role in assisting him/her to regain control by utilization restraints. Attempt to persuade the patient to be cooperative with the process.

3) At no time will the patient be assisted into or out of restraints by less than two nursing staff.

4) All harm full objects, i.e. shoes, jewelry, scarves, belts, shoelaces and matches etc must be removed before the restraints are applied.

5) .After administration of restraint patient will be kept under observation.

6) Attention will be paid during these observation to the patient's need for regular meals, bathing, toileting and fluids.

7) Once every 30 minutes the on duty nurse will assess the patient to determine that:

   a) Circulation is adequate
   b) Restraints are secure
   c) Body alignment is correct
   d) If clinically indicated, restraints are adjusted ,body realigned assuring the
   e) Above conditions are met.

8) Not less than every two hours , the physician and/or the assigned nurse must assess the patient for the purpose of determining whether continued restraints are necessary.

9) Vital Signs:

   a. Patient receiving intramuscular medication while being restarined will have their blood pressure (BP) and pulse assessed and documented half an hour after the administration of the IM psychotropic medication ,unless it's contraindicated by the patient's behavior.

   b. If the blood pressure and pulse cannot be assessed within that time frame due to patient's behavior, this fact must be documented in the record and an effort to obtain this data must be made at least by end of shift.

   c. If a patent is restrained for four hours or more,BP and P are to be taken every shft i.e every six hours for the duration of restraints.

   d.A physician must be notified of any significant change in the patients's BP or Pulse.

10) At the change of shift the incoming /outgoing nursing staff will assess and document the continuing need for restraint.

11) In the event of bruising or any other form of injury during the process of applying restraints, the same will be treated immediately accordingly and a record must be documented in the patients bed side ticket (BST).

H. Documentation:

The order for restraint shall:

1. Be written with date and time
2. Specify the type of restraint to be used
3. Be limited to four hours.
4. State the specific behaviors requiring restraints
5. Specify any special instructions or conditions of restraints
6. Be signed by the physician – incase of an emergency, the order must include the name of the doctor authorizing the restraint ,the name of the nurse obtaining the order.

Documentation during restraint shall include:

1. Patients behavior every 30 mins
2. Any medication ,treatment given to the patient during the time the patient was restrained ,include effectiveness.
3. Nurse to assess and document adequacy of circulation ,security of restraints and body alignment not less than every thirty minutes.
4. Offering of fluid to patient every two hours.
5. Oral hygiene/bath given.

If the patient has to be released from restraint, the condition of the patien justifying the reason to withdraw the patient from restraint has to be clearly documented by the physician and a summary/termination note must be documented.

I. Training of staff:

The administrative department of the hospital headed by the Chief Medical Superintendent shall provide education and training inclusive of an assessment of skills, to ensure clinical competency, minimization of the use of restraint and the safety of the process

J. Education and training shall be provided:

a. During the orientation for all new nursing staff (fresh recruit/transferred)

b. On going via scheduled training sessions.